

HOW TO OPEN A GROUP HOME BUSINESS in Virginia

Sponsored residential & group homes under the DD Waivers — serving adults with developmental disabilities.

A complete, plain-language, step-by-step startup guide — written so you can actually DO each step.

Figures current for 2026 · Every contact, rate & fee should be re-verified before you rely on it.

The Virginia model — read this first

“Group home” in Virginia means providing residential support to adults with developmental disabilities (DD), and the Commonwealth funds it through its three DD Waivers, run jointly by two agencies. Two things make Virginia distinctive, and they run through everything below. First, **TWO AGENCIES SHARE THE JOB**: DBHDS — the Department of Behavioral Health and Developmental Services — licenses providers, and DMAS — the Department of Medical Assistance Services (Medicaid) — enrolls and pays them. You deal with both. Second, residential support comes in two flavors: **SPONSORED RESIDENTIAL**, the host-home model where an individual lives in your home under a licensed sponsoring agency, and **GROUP HOME RESIDENTIAL**, where individuals live in a home your own licensed agency operates and staffs. Which one you open decides how much of the licensing you do yourself.

The two ways in. *Path A — become a sponsored residential provider. An individual with a DD lives in your home and shares family life; a DBHDS-licensed sponsoring agency recruits, trains, certifies, and supports you and does the billing. Smallest, fastest — you don't need your own DBHDS license. Path B — become your OWN DBHDS-licensed agency. You get licensed by DBHDS, enroll with DMAS, and run group home residential (and/or sponsor residential homes yourself). A much bigger lift — but it's the real business. This guide walks both, and is honest about which is realistic when.*

The names you'll deal with. *DBHDS = Department of Behavioral Health and Developmental Services (its Office of Licensing licenses providers; it oversees DD services). DMAS = Department of Medical Assistance Services / Medicaid (you enroll here after DBHDS licenses you, and it pays the claims). The DD Waivers = Community Living (CL), Family & Individual Supports (FIS), and Building Independence (BI) — residential supports are funded mainly through CL. CSB / BHA = your local Community Services Board or Behavioral Health Authority — the front door for eligibility and support coordination. Support Coordinator / Case Manager = the CSB staffer who builds the person's plan (the ISP). CRC = the DBHDS Community Resource Consultant who guides new providers.*

How to use this guide

Read it once start to finish, then use it as a checklist and a phone list. It's written for someone who has never opened a home — every time we name a thing, we stop, break it down, and tell you exactly where to go and what to ask. It is general information, not legal, tax, or billing advice; requirements, rates, and codes change, so verify each step with DBHDS, DMAS, your CSB, your fire marshal, and qualified local counsel (see Part 12) before you rely on it.

The journey at a glance

These milestones take you from idea to your first payment — whether you start as a sponsored home or build your own licensed agency.

1	Decide your path & set up the business	Path A (sponsored) or Path B (licensed agency); entity, EIN, NPI, insurance (Parts 3–4)
2	Learn the licensing standards	Read 12VAC35-105 and the DD Waiver

		rules; connect with your DBHDS CRC (Part 5)
3	Get licensed by DBHDS	Letter of Intent, Human Rights approval, and the Initial Provider Application (Part 5)
4	Enroll with Medicaid (DMAS)	Become a Virginia Medicaid provider after DBHDS licenses you (Part 5)
5	Ready the home	HCBS settings rights, licensing, and fire safety (Part 6)
6	Staff & write your policies	Direct support professionals, RN oversight, documentation (Parts 7–8)
7	Get a person placed	Waitlist, waiver slot, SIS assessment, CSB match & ISP (Part 9)
8	Deliver services & get paid	Bill DMAS through WaMS against the authorized ISP (Part 10)
9	Build & keep your census	Costs, timeline, and the relationships that fill homes (Part 11)

Rule of thumb. Start smaller than you think. Sponsored residential — one person in your home under a sponsoring agency that holds the license and does the billing — is the fastest, lowest-cost way in, and it's how many Virginia providers learn the system before they take on their own DBHDS license. You can grow into your own agency later, once you know the terrain.

Before you begin — a realistic picture

Two honest truths will save you time and money. First, this is a relationships-and-documentation business funded by Medicaid, not a real-estate play or a normal small business: your revenue depends on getting licensed, enrolled, and known — and then on a person's team choosing you, because individuals have freedom of choice among providers. So your reputation with DBHDS, your CSB, sponsoring agencies, and Support Coordinators is your most valuable asset. Second, demand is real but arrives one person at a time: Virginia's DD Waiver waiting list is large — more than 14,000 people — and the Commonwealth has been funding new waiver slots each year, but a person only reaches you when a slot opens, their ISP is built, and they choose you. DBHDS licensing and DMAS enrollment take months, and you don't bill until someone is placed and authorized — so you need patience and a working-capital cushion to carry any staff and setup costs through the ramp.

Here's how the rest of this guide is organized: Parts 1–2 explain what the business is and who regulates and pays you; Part 3 sets up the business itself, with the EIN, NPI, bank account, and insurance broken all the way down; Part 4 walks the two paths in; Part 5 is the DBHDS-licensing and DMAS-enrollment chain; Part 6 covers the home, HCBS rights, and fire safety; Parts 7–8 are staffing and the policies and records DBHDS licenses; Part 9 is how a person actually gets placed with you; Part 10 is how you get paid; Part 11 covers costs, timeline, and getting placements; and Part 12 collects every phone number and script in one place. Read it once end to end, then keep it beside you as a checklist.

PART 1

What This Business Is

In short: *Providing residential support to adults with developmental disabilities under Virginia's DD Waivers — as a sponsored residential home in your own house, or as a group home your agency operates. Paid by Medicaid (DMAS) after DBHDS licenses you.*

A “group home” business in Virginia is really a developmental-disabilities support business. You help an adult with a developmental disability live successfully in the community — with daily support, supervision, skill-building, help with health and medications, and a real home life — rather than in an institution. Virginia's DD Waivers fund a spectrum of residential support: Sponsored Residential (an individual lives in your home and shares family life, under a sponsoring agency); Group Home Residential (individuals live in a home your agency operates and staffs); and the more independent Supported Living Residential (in apartments). Residential supports are funded mainly through the Community Living (CL) waiver.

Be clear-eyed about what kind of business this is. This is a human-services business built on relationships and documentation, funded by Medicaid — not a real-estate play. Your value is the quality and reliability of the support you provide and your ability to keep it compliant and well-documented, because that's what DBHDS licenses, what DMAS pays for, and what keeps people safe. Go in understanding that the care and the paperwork ARE the business.

Why this business — and why now

Demand for community-based services for people with developmental disabilities is high and durable: Virginia, like every state, is moving people out of institutions and into the community, families overwhelmingly want a real home for their loved one, and the DD Waivers fund exactly this. Virginia's waiting list — more than 14,000 people — tells you the demand is real, and the Commonwealth funds new slots each year, so a reliable, well-run provider that builds relationships with CSB Support Coordinators can keep its homes full. And you can start lean — supporting one person as a sponsored residential provider under a licensed agency — and grow into your own licensed agency as you learn the system.

Be honest about the two realities that trip up new providers. (1) Two agencies, two steps: you don't just get one license and bill Medicaid — DBHDS licenses you AND DMAS enrolls and pays you, and each takes its own time. (2) Room and board is separate from your pay: the waiver pays for the SERVICE, not the person's rent and food, which come from their own income (usually SSI). Plan your business — and explain it to families — around both.

PART 2

Who Regulates You & Who Pays You

In short: *Two state agencies stand between you and a paid claim: DBHDS licenses you and DMAS enrolls and pays you. Your local CSB is the front door for eligibility, and its Support Coordinator builds each person's plan and matches them to a provider.*

DBHDS — Department of Behavioral Health and Developmental Services (your licenser)

DBHDS, through its Office of Licensing, licenses providers under 12VAC35-105 and oversees DD services across the Commonwealth. Without a DBHDS license you cannot operate a group home (Path B); on Path A, your sponsoring agency holds the DBHDS license. DBHDS also runs the Office of Human Rights (which approves your human-rights policies) and the Office of Provider Network Supports, whose regional Community Resource Consultants (CRCs) guide new providers and run the Provider Readiness and Education Program (PREP).

DMAS — Department of Medical Assistance Services / Medicaid (your payer)

After DBHDS licenses you, you enroll with DMAS as a Virginia Medicaid provider so you can be reimbursed for waiver services. DMAS sets the DD Waiver rates, publishes the fee schedule at [dmas.virginia.gov](https://www.dmas.virginia.gov), and runs the Waiver Management System (WaMS) that handles service authorizations and billing. On Path A, your sponsoring agency handles the DMAS billing.

The CSB — your local Community Services Board (the front door)

Every community in Virginia has a Community Services Board (or Behavioral Health Authority). The CSB screens people for eligibility, manages the DD Waiver waiting list locally, and employs the Support Coordinators (case managers) who develop each person's Individual Support Plan (ISP) and coordinate services. Because individuals have freedom of choice among providers, the Support Coordinator is the relationship that brings you placements.

What this means for you: the path to payment is a chain — DBHDS (license) → DMAS (enroll) → a person placed and authorized through their CSB — and each link takes time. Start them in the right order and in parallel where you can, and treat your DBHDS CRC and your CSB Support Coordinators as key relationships. A home that's licensed, enrolled, and known to the Support Coordinators in your area is a home that fills.

PART 3

Form Your Business & Insure — Step by Step

In short: *This is where most first-timers get stuck. We break each piece all the way down — what it is, where to go, exactly what to do, and what to have in hand.*

3.1 Form your legal entity (an LLC)

An “entity” is the legal business — almost always a Limited Liability Company (LLC) — that holds your license or registration, your contracts, and your bank account, and that shields your personal assets if the business is ever sued. In Virginia you form the LLC through the State Corporation Commission (SCC). Your entity, EIN, and NPI all have to be in place and consistent before DBHDS licensing and DMAS enrollment, because they're checked along the way. You'll name a “registered agent” (a person or service with a physical in-state address who receives legal mail — this can be you) and adopt an Operating Agreement (the internal document that sets out who owns what and who can make decisions).

- 1. File your Articles of Organization with the Virginia State Corporation Commission.** Online at the SCC's Clerk's Information System (cis.scc.virginia.gov) — the LLC filing fee is \$100. Name the LLC and list a registered agent (this can be you, or a Virginia resident/qualified business with a Virginia address).
- 2. Adopt an Operating Agreement.** The internal document that sets ownership and management — your bank, your DBHDS license file, and DMAS enrollment will reference your structure.
- 3. Pay the \$50 annual registration fee.** Virginia LLCs pay a \$50 annual registration fee to the SCC each year to stay in good standing (there's no separate annual report to file — it's the registration fee).
- 4. Confirm local business licenses.** Check what city or county business license (BPOL) and zoning approvals apply where you'll operate.

No newspaper publication in Virginia. Virginia has no LLC publication requirement — forming the entity is a straightforward SCC filing (\$100) plus the \$50 annual registration fee. That's simpler and cheaper than states that make you publish.

3.2 Get your EIN — the free federal tax ID

An EIN (Employer Identification Number) is your business's federal tax number — think of it as a Social Security number for the company. It's free, takes about ten minutes, and you need it before you can open a bank account, run payroll, or enroll with any payer. Here's exactly how to get it:

- 1. Go to [IRS.gov](https://irs.gov) and open the EIN Assistant.** Search “Apply for an EIN online.” Apply directly on the IRS website — never pay a third-party site that charges for this free service.
- 2. Choose your entity and enter the responsible party.** Select “Limited Liability Company,” then give the responsible party's legal name and SSN or ITIN (usually you, the owner), plus your business name and address.
- 3. Finish in one sitting.** The online session times out after about 15 minutes of inactivity and can't be saved partway — have your details ready before you start.
- 4. Save your EIN and the CP-575 letter.** You get the EIN on the spot and can download the official CP-575 confirmation letter — save both; your bank and every payer will ask for them.

3.3 Get your NPI — the national provider ID

An NPI (National Provider Identifier) is a free, ten-digit ID number that health-care payers use to identify your organization. You'll use it to enroll with DMAS as a Virginia Medicaid provider and to bill through WaMS for DD Waiver services. Your AGENCY needs a Type 2 (Organizational) NPI — which is different from the Type 1 (individual) NPI a single person would get.

- 1. Create an I&A account.** At nppes.cms.hhs.gov, set up an Identity & Access (I&A) account for whoever will manage the number — usually an owner.

2. Start a Type 2 (Organizational) application. Log into NPPES and choose Type 2, not Type 1. Type 1 is for an individual person; your business is a Type 2 organization.

3. Enter your legal name, EIN, and address, and pick your taxonomy. Choose the taxonomy that fits a developmental-disabilities / community-based residential provider — confirm the exact one DMAS wants. The “taxonomy” is the code that says what kind of provider you are — confirm the exact one your Medicaid program wants before you submit.

4. Submit — it's free and quick. It usually processes in about 10 business days. Save the NPI; you'll use it on every enrollment and claim.

3.4 Open a business bank account

Keep the business's money completely separate from your personal money — it protects your liability shield and makes payroll, taxes, and payer deposits clean. Open the account once your entity and EIN are set up.

What to bring to the bank:

- Your EIN confirmation (CP-575) from the IRS
- Your filed Articles of Organization / Certificate of Formation and the entity ID number
- Your signed Operating Agreement
- A Beneficial Ownership form (the bank collects details on anyone owning 25%+ plus a control person)
- A government photo ID, SSN, date of birth, and home address for each owner/signer
- An opening deposit (often around \$100)

Where to go: a bank with lots of Virginia branches — Atlantic Union Bank, Truist, Wells Fargo, TowneBank, or a local Virginia credit union. Home care is payroll-heavy — you pay caregivers weekly, but Medicaid and insurers pay you weeks later — so ask specifically about a business line of credit or invoice financing to bridge payroll before you scale.

3.5 Get your insurance — what you need, who sells it, and what to ask

A DD residential provider brings staff into people's homes and supports vulnerable adults, so you need coverage built for a human-services provider — and DBHDS and DMAS will expect proof:

Coverage	What it protects against	Must-have?
General & Professional Liability	Injury or property damage, plus claims about the SUPPORT and supervision you provide	Yes — foundational
Abuse & Molestation	Allegations of abuse of a person you support — general liability EXCLUDES this	Yes — high exposure
Directors & Officers / Management	Regulatory, employment, and governance claims	Recommended for an agency
Non-Owned & Hired Auto	DSPs driving their own cars to support or transport the people you serve	Yes — critical
Workers' Compensation	DSP injuries	Required in VA (2+ employees)
Property	Coverage on any provider-owned or leased group home	If you own/lease a home

Abuse & molestation is your highest exposure. Supporting vulnerable adults means abuse & molestation coverage — EXCLUDED by standard general liability — is essential, with a real limit, not a token sub-limit. Confirm it explicitly.

Real places to get it

Buy through an independent agent or broker who writes HUMAN-SERVICES / developmental-disabilities provider programs — this is a specialty niche. Carriers and programs active in DD/IDD services include NSM/Irwin Siegel Agency (a social-services specialist), Negley Associates, Philadelphia Insurance (PHLY), and CNA. Find a local Virginia agent at [trustedchoice.com](https://www.trustedchoice.com) and ask specifically for a developmental-disabilities / DD-waiver provider program. Confirm the carrier writes this in Virginia before you rely on a quote.

What to ask the insurance agent

“Do you write developmental-disabilities / residential providers in Virginia — which carriers, and what's the minimum premium?”

“Is abuse & molestation included, at what limit, and is it a sub-limit or a full limit?”

“Do you include non-owned & hired auto for my DSPs driving the people we support?”

“What limits do DBHDS and DMAS expect?”

“Can you set up Virginia workers' comp for my direct support professionals?”

3.6 Keep your paperwork consistent — every step downstream checks it

One practical warning that saves Virginia providers real time: your legal name, ownership, EIN, and NPI have to line up EXACTLY across every step that follows — your DBHDS Initial Provider Application, your Human Rights policy submission, and your DMAS Medicaid enrollment. Your information passes through both agencies before a claim pays, and a mismatch at any handoff — a slightly different business name here, an old address there, or a P.O. box where a real street address is required — is one of the most common and most frustrating causes of a stalled application or a rejected claim. (Remember: DBHDS requires a real street address for your licensed location; a P.O. box won't satisfy it.) Set the entity up once, correctly, and make every downstream document point at the same clean legal picture. It's worth an hour with your accountant or attorney up front to confirm the structure, because untangling a mismatch after you've applied costs far more than getting it consistent now. And keep your DBHDS license, your Human Rights approval, and your DMAS enrollment calendared for renewal — letting any one of them lapse stops your payments even when the care is going perfectly.

The business foundation to have in hand before you apply for licensing:

- Your formed Virginia LLC (Articles of Organization filed, \$50 annual registration current) and Operating Agreement
- Your EIN confirmation (CP-575) from the IRS
- Your Type 2 (organizational) NPI from NPPES
- A business bank account opened in the entity's exact legal name
- Bound insurance — general/professional liability, abuse & molestation, non-owned auto, and workers' comp
- A single, consistent legal name, ownership list, and real street address used identically on every document

With those six things in place and consistent, you're ready to build the DBHDS application in Part 5 on a clean foundation — and you've removed the paperwork mismatches that stall more Virginia applications than any substantive problem does.

PART 4

The Two Paths — Sponsored Residential (A) or Licensed Agency (B)

In short: Path A is the fast, small way in — become a sponsored residential provider under a DBHDS-licensed sponsoring agency. Path B is the real business — become your own DBHDS-licensed agency. Pick the one that fits where you are today; many start with A and grow into B.

4.1 Path A — a sponsored residential home

Sponsored residential means an adult with a developmental disability lives in your home and shares your everyday life. You don't get licensed yourself — you partner with a DBHDS-licensed agency that sponsors residential homes; they recruit, train, and certify you, support you day to day, and handle the Medicaid billing. It's the fastest way in and the closest thing to a host home.

- 1. Make sure it fits your life.** Everyone in the household is part of this, so it's a family decision — go in with eyes open about the good days and the hard ones.
- 2. Choose a DBHDS-licensed sponsoring agency.** Interview more than one — how they certify, train, support, and pay sponsored providers varies a lot. They hold the license and do the billing; you provide the home and the day-to-day support.
- 3. Get certified & trained.** The agency completes your certification (using DBHDS's Sponsored Residential Certification process) and your training, and makes sure your home meets the HCBS settings requirements. This is the heart of sponsored-residential approval.
- 4. Pass background checks, then get matched.** You and the adults in your home complete the required criminal-background and registry checks; then the agency, the person's Support Coordinator, and the team match you with someone whose needs and personality fit your home — and the person has freedom to choose.

***The difficulty-of-care tax note.** Because the person you support lives in your OWN home, your sponsored-residential payments may qualify for the IRS "difficulty-of-care" exclusion (potentially non-taxable). Confirm with a tax professional — it can meaningfully change the economics of a sponsored-residential arrangement.*

4.2 Path B — become your own DBHDS-licensed agency

Path B is more work, more money, and more control: you get licensed by DBHDS, enroll with DMAS, and run group home residential (and/or sponsor residential homes yourself). It's the real, scalable business — and Part 5 walks the licensing path step by step. Most operators who go this route either have direct experience in DD services or hire someone who does, because your application has to show DBHDS you understand the standards and can deliver.

PART 5

Get Licensed by DBHDS & Enrolled by DMAS

In short: For Path B, this is the chain that unlocks payment: connect with your CRC and learn the standards, submit your Letter of Intent and Human Rights policies, apply for your DBHDS license through the CONNECT portal, then enroll with DMAS as a Medicaid provider.

- 1. Set up the business & connect with your CRC.** Form your entity (Part 3) and get your EIN and NPI. Then reach out as early as possible to your regional Provider Team Community Resource Consultant (CRC) in the DBHDS Office of Provider Network Supports — they guide new providers. Attend the Provider Readiness and Education Program (PREP) / licensing orientation.
- 2. Submit your Letter of Intent & Human Rights policies.** Submit a Letter of Intent (LOI) to DBHDS — the services you'll provide, the population, and your locations (a P.O. box won't satisfy the address requirement). Develop policies that comply with the Human Rights Regulations, and email your Complaint Resolution Policy and Human Rights Compliance Verification Checklist to the Office of Human Rights (OHRPolicy@dbhds.virginia.gov) for approval.
- 3. Apply for your DBHDS license (12VAC35-105).** Submit your Initial Provider Application and all required attachments through the DBHDS CONNECT Provider Portal, and demonstrate knowledge of the required licensing policies, procedures, and forms. DBHDS is currently prioritizing initial applications for the services most needed across the Commonwealth.
- 4. Enroll with DMAS (Medicaid).** After you're licensed by DBHDS, enroll as a Virginia Medicaid provider through DMAS so you can be reimbursed for waiver services — you'll need your NPI and business information, and you'll use the Waiver Management System (WaMS) for authorizations and billing.
- 5. Meet the HCBS settings standard, then staff & serve.** Every group home, sponsored residential, supported living, and group day setting must fully comply with the federal HCBS settings requirements. Put your policies in place (Part 8), hire and train your DSPs (and complete the DSP Competencies Checklist within 180 days), then accept referrals, deliver services, and bill. Ask your CRC about Jump Start grants for new or expanding providers.

What this means for you: licensing is a quality gate, not a form. DBHDS wants evidence — real policies, real service descriptions, real capacity — that you can safely support people. The providers who get through cleanly treat 12VAC35-105 and the DD Waiver rules as the blueprint for the whole operation, so building your policies and staffing to the standard does double duty: it earns your license AND it's how you'll actually run.

PART 6

The Home — Settings, HCBS Rights & Fire Safety

In short: A sponsored residential home is a private family home, certified by your sponsoring agency. A group home is DBHDS-licensed under 12VAC35-105 and inspected. In every case it must meet the federal HCBS settings rules and genuinely feel like the person's home.

6.1 How Virginia handles the home

A sponsored residential home is a private family home, certified by your sponsoring agency and meeting the HCBS settings requirements — it's a home, not a licensed facility you hold. A group home is licensed by DBHDS under 12VAC35-105 and inspected, and must also meet the HCBS settings requirements in every setting. In both, the home must genuinely feel like the person's home — privacy, a lease or legally enforceable residency agreement, choice, and full access to the community.

6.2 “Do I need fire sprinklers?” — the honest answer

A sponsored residential home (a private family home) is held to residential standards, so a full commercial fire-sprinkler system generally is NOT required. A group home's fire requirements come from the Virginia Statewide Fire Prevention Code and the building code and depend on the home's size, occupancy, and the residents' abilities — so a larger or licensed group home may face requirements a family home doesn't. Because it's set by the fire code and your local fire marshal, confirm your specific home with the local fire marshal and building official BEFORE you buy or renovate.

6.3 The home checklist

- ☐ Sponsored residential: certification by your sponsoring agency and HCBS-settings compliance. Group home: a DBHDS license (12VAC35-105) and inspection.
- ☐ HCBS settings rights: a lease or legally enforceable residency agreement, privacy, choice, and full community access — in every setting.
- ☐ Working smoke and carbon-monoxide detectors, extinguishers, clear exits, and a practiced emergency plan; fire-safety sign-off appropriate to the setting.
- ☐ A safe, clean, sanitary home — sound utilities and electrical, working plumbing, and safe food storage and preparation (a VDH health inspection may apply if you serve food).
- ☐ A real street address for the licensed location (a P.O. box won't meet the requirement).

Two things people get wrong. (1) *Sponsored vs. licensed changes everything — for sponsored residential you don't hold a DBHDS license (your sponsoring agency does); for a group home, you do. Pick your path before you spend money.* (2) *Room and board is separate from your pay — the waiver pays for the service, not the person's rent and food (those come from their own income, usually SSI). Keep the two completely separate in your books and your agreements.*

PART 7

Who You Actually Need — Staffing

In short: *On Path A, you and your household provide the support, with your sponsoring agency's professional oversight, certification, and billing — you don't build a staff. As a licensed agency (Path B), plan for a director, direct support professionals, nurse oversight, a DSP supervisor, and billing — one person can wear several hats at the start.*

7.1 The roles

Sponsored residential (Path A): you and your household are the support. The sponsoring agency provides the professional oversight, certification, and billing, and helps arrange relief/respice. You don't build a staff.

Group home agency (Path B), plan for these roles:

- **Agency director / administrator:** runs the agency, owns the DBHDS license and compliance, and holds the DMAS enrollment.
- **Direct support professionals (DSPs):** the caregivers in your homes — each must meet the DSP Competencies within 180 days of hire.
- **DSP supervisor:** oversees the DSPs and confirms competencies.
- **Registered Nurse:** to oversee certain medical supports and medication — often part-time or contracted at first.
- **Billing / administrative support:** handles DMAS billing through WaMS and keeps documentation current.

7.2 Recruiting and keeping DSPs — the real make-or-break

Direct support professionals are the heart of a group home and the hardest thing to keep. DSP turnover is high across the whole field, and a home that can't reliably staff its shifts can't safely support people or keep its authorizations — so treat recruiting and retention as your core operation, not an afterthought. (On Path A this is your sponsoring agency's job; on Path B it's yours.)

- **Recruit continuously and hire for heart:** specific tasks can be trained; patience, reliability, and genuine care for people with disabilities cannot. Check references for dependability.
- **Onboard to the standard:** real orientation, the required background and registry checks and training, and the DSP Competencies checked (and documented) within 180 days before a DSP supports someone alone — this protects the people you serve and satisfies DBHDS.
- **Retain deliberately:** consistent schedules, correct and on-time pay, real support when a DSP hits a hard day, and recognition. Every DSP who quits costs you re-hiring, re-training, and often continuity for the person they supported.

PART 8

Policies, Records & Staying Compliant

In short: *Your policies and documentation are what DBHDS licenses and what DMAS pays against. Build them to the 12VAC35-105 and Human Rights standard from day one — medications, emergencies, incident reporting through CHRIS, and the rights of the people you support.*

You keep clear documentation covering medications, emergencies, incident reporting, and the rights of the person you support — and the person's Individual Support Plan (ISP) drives what you deliver and record. Virginia's Human Rights Regulations require a working system for protecting rights and resolving complaints, and serious incidents are reported through the DBHDS Computerized Human Rights Information System (CHRIS). This documentation is both the licensing requirement and the backbone of a safe, well-run operation. Build a simple system that keeps each person's ISP, service records, and incident documentation current, because DBHDS reviews it and DMAS can audit it.

8.1 Documentation & staying inspection-ready

Whatever your state requires, the agencies that pass surveys and payer audits cleanly all do the same thing: they keep complete, current files from day one instead of scrambling the week before an inspection. Build these habits now, because reconstructing records after the fact is nearly impossible:

- **A file for every caregiver:** the application, background checks, training and competency records, health/TB screening, and a signed acknowledgment of your policies — all dated and kept current.
- **A file for every client:** the assessment, the service/care plan, the signed service agreement, consents, ongoing visit notes, and any payer authorizations.
- **A visit record for every shift:** who worked, when they arrived and left, and what care was provided — and, for Medicaid, the matching EVV record.
- **An incident and complaint log:** every incident, injury, and complaint and exactly how you resolved it — inspectors look for a real, used quality-improvement process, not a binder that's never been opened.

PART 9

The Person's Journey — Waitlist to Move-In

In short: You can't get paid until someone is placed with you and their waiver services are authorized. The path runs from eligibility through the CSB, across Virginia's large waiting list, through a waiver slot and the SIS assessment, to the ISP and the match by the Support Coordinator.

- 1. The person qualifies — and waits.** They're screened and assessed for a DD Waiver through their local CSB. Because slots are limited, most people spend time on Virginia's waiting list (more than 14,000 statewide) before a slot opens.
- 2. They get a waiver slot.** When a slot is available, a Waiver Slot Assignment Committee (WSAC) recommends who receives it (Priority 1 first).
- 3. The plan and level are set.** A Support Coordinator develops the person's ISP, and a Supports Intensity Scale (SIS) assessment helps set the support-need level — and the reimbursement tier — for residential services.
- 4. The match.** The person chooses providers (freedom of choice); you (or your sponsoring agency) are matched based on fit, your certification/license, and your capacity.
- 5. Move-in.** With the ISP and authorization in place (in WaMS), the person moves in and you're delivering a billable service.

What this means for you: because members choose their provider and slots are limited, your census depends on relationships and reputation, not marketing. Get known to DBHDS, to sponsoring agencies, and above all to CSB Support Coordinators as the provider who supports people well, communicates, and handles a placement smoothly — and the matches follow.

***Plan your business around the waitlist reality.** Virginia's waiting list is long — more than 14,000 people — and slots open on their own schedule through the WSAC process, which shapes how you should plan. Don't build a business model that assumes you'll fill every bed the month you're licensed; assume a gradual build as Support Coordinators learn to trust you and matches come through. Two practical moves help: start on Path A as a sponsored residential provider under an established agency so you have revenue while you build, and stay in regular, helpful contact with the Support Coordinators at the CSBs in your area so that when one of their members is choosing a provider, you're the name they think of first. Patience plus reliability is the whole strategy — the demand is real, but it arrives one authorized person at a time.*

PART 10

How You Get Paid

In short: Two keys unlock payment — a DBHDS license and DMAS Medicaid enrollment — and DMAS pays a residential rate driven by the person's assessed support-need level. Room and board is separate.

10.1 How the rate works

Residential supports are paid through Virginia's Medicaid DD Waiver. The rate is driven by the person's assessed support-need level — the Supports Intensity Scale (SIS) sorts people into levels that map to reimbursement tiers, so a person with higher needs carries a higher rate. Sponsored residential is generally paid monthly; group home residential is paid per the waiver's rate structure for the person's tier.

- **Support-need level / tier:** the SIS-based level is the biggest driver of the residential rate — higher needs, higher reimbursement.
- **Sponsored vs. group home:** the two services are priced differently; a staffed group home carries different costs and a different rate than a sponsored home.
- **Where to look:** exact procedure codes and current rates are in the DMAS DD Waiver fee schedule / rates (dmas.virginia.gov). Confirm the current figure before you budget.
- **Room and board is separate:** the waiver pays the service, not the person's rent and food (those come from their SSI / personal income).

10.2 The money math — how residential pencils out

Residential economics are different from an hourly private-pay business — your revenue is the waiver rate for the authorized tier, and your biggest cost is DSP labor. Lay it out with your own numbers:

- **Revenue:** the authorized residential service in each person's ISP, billed to DMAS through WaMS at the waiver rate for their SIS-based tier — plus each additional resident in a group home.
- **Cost:** DSP wages and payroll taxes are the bulk; then RN oversight, administration, insurance, and (for a group home) the building. Most are fixed against your authorized census.
- **The lever:** because the rate is set by the waiver and the tier, your margin comes from staffing efficiently to each person's authorized plan and keeping your home filled with authorized people — utilization, not rate, is the number you manage.
- **Room and board stays separate:** the person's rent and food come from their own income (usually SSI), NOT the waiver — keep that money and those agreements entirely separate from your service billing.

10.3 Your first payment

1. **Confirm the service and tier are authorized.** Make sure the residential service and tier are in the person's ISP and in WaMS.
2. **Deliver the service and keep your documentation.** Keep DSP competencies current — billing must stop if they lapse.
3. **Submit the claim through DMAS / WaMS.** Your sponsoring agency does this for a sponsored home; a group home bills per its DMAS enrollment.
4. **Get paid — and keep everything current.** Keep your authorizations and your license/certification current so payment never lapses.

PART 11

What It Costs, How Long It Takes & Getting Placements

In short: *Path A can start relatively quickly; Path B takes several months through PREP, the Letter of Intent, Human Rights approval, the license, and DMAS enrollment. Placements come through the CSB, so your relationships with Support Coordinators are your census.*

11.1 Timeline & startup costs

A sponsored residential home (Path A) can move relatively quickly once you've chosen a sponsoring agency — certification, training, and background checks. Becoming your own DBHDS-licensed agency (Path B) takes longer: the business setup, PREP/orientation, your Letter of Intent, Human Rights approval, the Initial Provider Application and license, and DMAS enrollment — plan for several months end to end. Sponsored-residential costs are modest (mainly your time, background checks, training, and getting your home ready); a licensed agency adds business formation, insurance, staff payroll, the licensing process, and your policies and procedures — though the DBHDS Jump Start grant program can help new or expanding providers. Confirm current fees and timelines with your CRC, DBHDS Licensing, and DMAS.

11.2 Getting your first placements

Placements don't come from advertising — they come through the CSB process and your reputation. Here's a simple plan:

- 1. Get known to DBHDS and sponsoring agencies.** Connect with your regional DBHDS CRC and — if you're starting on Path A — interview the sponsoring agencies that recruit and support sponsored residential providers in your area.
- 2. Build relationships with CSB Support Coordinators.** They develop the ISP and, with the person's choice, match members to providers. Be the provider they trust to handle a placement well, communicate, and never leave a shift unstaffed.
- 3. Be responsive and reliable.** Answer the phone, say yes to a good match, handle move-in smoothly, and follow through — one good placement earns the next referral.

PART 12

Who to Call & Exactly What to Ask — Every Verify, in One Place

In short: DBHDS and your local CSB are the center of gravity, with DMAS and your fire marshal close behind. Statewide contacts are the same for everyone; your CSB and fire/building offices are local. Here's the list and the scripts.

12.1 Statewide (same for everyone)

For...	Contact
Becoming a licensed provider	DBHDS Office of Licensing — CONNECT Provider Portal · P.O. Box 1797, Richmond, VA 23218
Provider help & getting started	Your regional Provider Team CRC — DBHDS Office of Provider Network Supports (PREP class)
Human Rights policy approval	DBHDS Office of Human Rights — OHRPolicy@dbhds.virginia.gov
Medicaid enrollment & rates	DMAS — dmas.virginia.gov (Provider Enrollment; DD Waiver fee schedule; WaMS)
Waiver info & finding your CSB	My Life My Community — mylifemycommunityvirginia.org
Business, EIN & NPI	cis.scc.virginia.gov · irs.gov (EIN) · nppes.cms.hhs.gov (NPI)

12.2 The local calls — and exactly what to ask

① Your local CSB or a sponsoring agency — your most important calls

"I want to become a sponsored residential provider / open a group home. Who sponsors residential homes here, and what's the current need?"

"(To a sponsor) How do you certify, train, and support sponsored providers, how do you match, and how and when do you pay?"

② Your DBHDS CRC — the licensing path

"I want to start a DD Waiver group home. Can you walk me through PREP, the Letter of Intent, and the initial application in CONNECT?"

③ Local fire marshal / building official — the home

"I'm opening a sponsored residential home / a licensed group home for adults with disabilities at this address. What fire-safety requirements apply, and do I need sprinklers?"

12.3 Worked examples — two of Virginia's biggest markets

DBHDS Licensing, DMAS enrollment, and My Life My Community are statewide — the local piece is your CSB and your fire marshal. Virginia has a CSB for every community; find yours by where you live.

- **Northern Virginia (Fairfax):** Fairfax-Falls Church Community Services Board is your entry point for screening and support coordination (Fairfax County is Virginia's most populous area). DBHDS Office of Licensing (Northern region) and your regional CRC; DMAS for enrollment; Fairfax County (or your city/town) fire marshal and building official for a group home. The quirk: it's the state's largest, highest-cost market — strong demand, but plan for higher real-estate and staffing costs.
- **Virginia Beach (Hampton Roads):** Virginia Beach Community Services Board is your entry point (Virginia Beach is the state's most populous city). DBHDS Office of Licensing (Eastern region) and your regional CRC; DMAS for enrollment; City of Virginia Beach fire marshal and building official for a group home. The quirk: the Hampton Roads region spans several cities — confirm which city's CSB and fire marshal cover your specific address.

Common questions, answered

Do I have to take Medicaid?

No. Many successful agencies run entirely on private pay, long-term-care insurance, and veterans' benefits — it's often the smarter way to start, because you earn immediately without the enrollment, contracting, and EVV that Medicaid requires. You can add Medicaid later once you have cash flow and a team.

Can I run this from home at first?

Often yes for a small non-medical agency — the care happens in the client's home, so you may not need a storefront to start. Confirm your local zoning and your license/registration rules (some require a business address), and know you'll want real office space as you grow.

Should my caregivers be employees or independent contractors?

Almost always W-2 employees. Classifying caregivers as 1099 contractors to save on taxes is one of the most common — and most expensive — mistakes in home care; it invites wage-and-hour and tax liability, and most payers and insurers require employees. Treat them as employees and carry workers' comp.

How much cash do I really need to start?

Enough to cover several weeks — ideally a couple of months — of caregiver payroll before your first payer reimbursements arrive, plus your license/insurance/software setup. Under-capitalization is the number-one reason new agencies fail; line up a business line of credit before you scale.

How long until it's profitable?

Most agencies take several months to a year to build a steady client base and referral flow. Starting on private pay shortens the runway because you're paid quickly; Medicaid volume comes later and pays slower. Plan your finances for a ramp, not an overnight full census.

Do I need experience in health care?

It helps, but plenty of successful owners come from business, sales, or family-caregiving backgrounds and hire the clinical expertise they need (for example, a nurse where the state requires one). What matters more is being organized, responsive, good with people, and disciplined about compliance and cash.

What actually makes one agency win over another?

Reliability. Families and referral sources choose — and stay with — the agency that answers the phone, starts care fast, sends the same dependable caregiver, communicates, and never leaves a shift unstaffed. Get that right and referrals compound; get it wrong and no amount of marketing saves you.

Do I have to become a licensed agency, or can I start smaller?

You can start smaller — Path A lets you become a sponsored residential provider, hosting one person in your home under a DBHDS-licensed sponsoring agency that holds the license and does the billing. It's the fastest way in and the best way to learn the system before you take on your own DBHDS license (Path B). Many providers grow from A to B.

Why do I have to deal with two agencies — isn't this just Medicaid?

Virginia splits the job: DBHDS licenses providers and oversees DD services, while DMAS is Medicaid — it enrolls you and pays the claims. So the chain is DBHDS licenses you, then DMAS enrolls you, and a person is placed and authorized through their local CSB. You generally need all three moving parts.

Does the waiver pay for the person's rent and food?

No — the waiver pays for the SERVICE (the support you provide), not room and board. The person's rent and food come from their own income, usually SSI. Keep the two completely separate in your agreements and your books; mixing them is a common and serious mistake.

How long is the waiting list, and how does someone reach me?

Virginia's DD Waiver waiting list is large — more than 14,000 people — and the Commonwealth funds new slots each year. When a slot opens, a Waiver Slot Assignment Committee recommends who receives it, the CSB Support Coordinator builds the ISP and a SIS assessment sets the tier, and the person chooses a provider. Your job is to be known and trusted to the Support Coordinators in your area so you're the name they think of.

You can do this

Opening a group home business in Virginia is real work — two agencies, a license, and a waiver system — but it's a real, walkable path, and people across the Commonwealth do it every day, giving someone a real home in the community. You don't have to be a lawyer or hire a consultant. Whether you start with a sponsored residential home or build your own licensed agency, take it one step at a time: set up the business, learn the licensing standards, get licensed by DBHDS and enrolled by DMAS, ready the home, and build the relationships that bring you placements. Verify the current details with the Commonwealth as you go, and lean on your CSB and your DBHDS CRC — that's what they're there for.

Reminder. Prepared for PolicyReady.org and grounded in Virginia DBHDS and DMAS rules, 12VAC35-105 and 12VAC30-122, and the DD Waiver standards (Community Living, Family & Individual Supports, and Building Independence) as of 2026. This is general information, not legal, tax, or billing advice. Requirements, rates, and codes change — always verify current details with DBHDS, DMAS, your CSB, and qualified counsel before you act.